

Pacific Biomedical Network Meeting

Medical Equipment Donation Policy

Country

Ministry/Department of Health

Generic Donation Policy

Authorised by: _____

Authorised on: _____

Summary

Many developing countries rely on donor support to boost their limited resources for acquiring adequate medical equipment. Often the intended benefits of medical equipment donation are not realised. Effective policy is needed to maximise the benefit of donations and to avoid the burdens that have arisen through inappropriate donations and ineffective communications between the donor and recipient government.

The following principles should apply:

- There must be **effective** communication between donor and recipient throughout **all** stages of the donation process.
- There must be no double standard in quality between acceptability in donor and recipient country.
- Donations must be made in conformance with government policies and procedures in the recipient country.
- Equipment donations must maximise the benefit to the recipient country as much as possible.
- The recipient country should not utilize their own funds for any donation.

A donation plan is required that covers the following issues:

- Are trained personnel available to operate the equipment effectively?
- Are trained personnel available to check and maintain the equipment?
- Are support resources available such as manuals, reagents, consumables and spare parts?
- Does the equipment meet safety, compliance, expected useful life and appropriateness standards?
- Can the equipment be effectively installed and commissioned?
- Can any special requirements for air, steam, cooling, power, water quality, shielding etc. be met?

The donor should ensure that donated equipment is complete and tested as meeting the original specifications before shipment.

The recipient should ensure that Customs clearance, local transportation, site preparation, installation, commissioning, periodic checking, calibration and maintenance are undertaken by technically competent staff in accordance with good health care equipment practice.

Used medical equipment requires extra care in assessment of the possible benefits against the known risks of extra maintenance, availability of consumables and spare parts and lack of manufacturer support.

Refurbishment of used medical equipment must be carried out by competent organisations to the manufacturer's original specifications. Complete documentation must be supplied and defect liability insurance be provided.

Both donor and recipient should evaluate the success or failure of each donation to improve future success rates.

1.0 Introduction

The Donation Policy of *Ministry/Department of Health* describes the process for donation and acceptance of medical equipment to *Country* to ensure that equipment donated is appropriate to

Country's clinical needs and conforms to specified medical equipment standards set by the *Ministry/Department of Health*.

2.0 Background

Country like many other developing countries faces problems with meeting equipment needs for delivery of health services and welcomes donations from various organisations, hospitals and individuals. Though the donations are received with much appreciation, some equipment is left idle in health facilities due to one or more of the following reasons:

1. Equipment is not working
2. Equipment is of the wrong rating (voltage/frequency)
3. Essential parts and accessories are not included thus equipment is incomplete
4. Model is not the standard in the country and incurs additional cost to use/maintain it
5. The model is obsolete and no longer supported by the manufacturer thus unable to source parts/accessories to maintain/use equipment
6. No information on who to contact for service support
7. Equipment is not suitable for the environment it has been donated for
8. User and Service manuals are not included
9. Users and technicians do not know how to use and maintain equipment and no training has been offered with equipment

Furthermore, lack of communication between the two parties often leads to receiving of donations not known by key people in the *Ministry/Department of Health*. Equipment has been noted to be left in storage for long periods before it is installed only to realize that it is inoperable.

There are associated cost issues that may need to be considered such as installation costs, ongoing consumables and maintenance costs. These may not be budgeted for if the donation was not part of *Ministry/Department of Health's* plan to maintain clinical services and equipment.

Thus, a donation policy is required to address the issues highlighted above to ensure that healthcare services are not adversely affected.

3.0 Principles of Donation

The policy adheres to four (4) underlying principles forming the core of *Good Donation Practice* as set out by World Health Organisation (WHO)¹, which are:

1. Health care equipment donations should benefit the recipient to the maximum extent possible.
2. Donations should be given with due respect for the wishes and authority of the recipient, and in conformity with government policies and administrative arrangements of the recipient country.
3. There should be no double standard in quality. If the quality of an item is unacceptable in the donor country, it is also unacceptable as a donation.
4. There should be effective communication between the donor and the recipient, with all donations made according to a plan formulated by both parties.

4.0 Scope

This policy applies to the acceptance of donated medical equipment to all public health facilities in *Country*.

5.0 Donation Process

The donation process is attached at Appendix 1.

6.0 Pre-Donation Requirements

6.1 Responsibility of Donor

Where the potential donor is notified of the “*focal contact person of Ministry/Department of Health*”, it is the responsibility of the donor to contact “*the focal person*” to formalize the donation process before making any shipments.

The donor must respect the four core principles for donations as stated in this policy.

It shall be the responsibility of the donor to make a critical evaluation of the intended medical equipment to be donated and inform the *Ministry/Department of Health* all relevant information and answer any question *Ministry/Department of Health* may have about the equipment. The donor shall also advise if freight charges are to be paid for by *Ministry/Department of Health*.

The donor shall ensure that:

- Equipment is fully operational and all essential accessories, consumables and spare parts are available for immediate use of equipment
- Consumables and accessories are supplied for 1 year of normal use

¹ WHO, *Guidelines for Healthcare Equipment Donations*, March 2000, p.1

- Consumables, accessories and spare parts are available from the manufacturer for at least 5 years
- All relevant documentation such as installation requirements, safety guidelines, environmental conditions, required consumables and accessories, operating and service manuals, service checklists and any training aids are available and in English
- Where applicable, equipment is of the standard model required by the *Ministry/Department of Health*.

6.2 Responsibility of Recipient

It is the responsibility of *Ministry/Department of Health* to ensure all health departments and relevant stakeholders are aware of the donation policy.

In instances where a donor contacts a health facility or a clinician or any *Ministry/Department of Health* staff directly, it will be the responsible of the latter to direct the donor to “a focal person from the *Equipment Committee*” for a proper donation process to take place.

It shall be the responsibility of the “focal person” to justify the need for the medical equipment being requested for or being offered using the donation plan and collaboration with respective clinicians, clinical representatives, biomedical staffs, and the equipment committee and communicate effectively with the potential donor.

Where applicable, the *Ministry/Department of Health* shall identify to the donor, the standard model of equipment required based on the current model in use.

Where applicable, the *Ministry/Department of Health* will use information provided by the donor to prepare the site for installation and commissioning of equipment.

A donation plan must be in place which covers the following issues to be considered for every donation:

- The genuine need of equipment for provision of health care service
- Availability of trained personnel to operate equipment effectively
- Availability of trained personnel to check and maintain equipment
- Availability of support resources such as manuals, reagents, consumables and spare parts
- Does the equipment meet safety, compliance, expected useful life and appropriateness standards?
- Can the equipment be effectively installed and commissioned?
- Can any special requirements of air, steam, cooling, power, water quality, shielding, etc be met?
- Additional costs incurred from the acceptance of equipment

The *Ministry/Department of Health* will provide a written approval to the donor to acknowledge the acceptance of the donation.

7.0 Prioritising Donations

Ministry/Department of Health shall prioritise acceptance of donated equipment based on the priority needs of equipment required to provide health care services. Where equipment is required to be replaced, this will take precedence over additional equipment.

8.0 Obsolescence

It shall be the responsibility of the donor to ensure sales/technical support is available from the manufacturer for a **minimum of 5 years**. This support should include spare parts, consumables & accessories (reusable or disposable), troubleshooting, and repair and maintenance assistance.

Obsolete equipment or equipment without any sales/service support will **not** be donated to *Country* unless agreed by *Ministry/Department of Health* for reasons that will benefit the health services. In such cases, equipment donated will be clearly marked with the identified reason for donation.

9.0 Training

It is essential for users and service technicians to be trained on the operation and maintenance of equipment to be donated. If the equipment model is not a standard, *Ministry/Department of Health* must ensure that training is organized for both users and technicians. Should any difficulty be faced, the donor should suggest alternatives. Where possible, the donor can provide training to the users and technicians upon delivery of equipment.

Once satisfactory training has been received, the donor must be informed by the *Ministry/Department of Health* to send the equipment.

10.0 Assembly, Packaging, Shipping

The donor must ensure that equipment is tested and passed performance verification by a competent biomedical engineer or a biomedical service organisation as per the manufacturers' specifications prior to shipment. Refurbishment of used equipment must be carried out by competent organisations to the manufacturer's original specifications. Complete documentation must be supplied, and defect liability insurance be provided.

All parts, accessories and required documentations necessary for the operation must be packed and labelled accordingly. Where applicable, the following pre-cautions must be taken prior to packaging and shipment:

- Equipment containing patient material must be de-contaminated
- Radioactive sources must be removed and properly packaged in special containers (with radioactive marking on outside)
- Fluids should be drained and fragile parts packaged with great care

Equipment must be packaged in accordance to method of shipping to avoid damage in transit and shipping documents must state the contents of the packaging/container and that it is a donation.

11.0 Customs Clearance, Local transportation and Unpacking

Customs clearance and transportation to the final destination shall be the sole responsibility of *Ministry/Department of Health*. Any special documentation required to assist with clearance shall be requested from the donor in advance.

All packages shall be unpacked and checked for damage and contents as per packing list. Any irregularities or missing items shall be reported to the donor immediately.

12.0 Installation, Commissioning and Maintenance

Equipment shall be inspected by a technically competent person and included in the asset register, prior to installation. Where there are special installation requirements, installation shall take place in accordance to instructions received from the donor and done by a technically competent person.

Associated costs of installation and commissioning shall be the responsibility of the *Ministry/Department of Health* unless the donor agency is willing to meet the costs.

Equipment shall be commissioned by trained personnel in accordance with normal, standard principles for all health care services practice, prior to clinical use.

Ministry/Department of Health shall be responsible for having a maintenance programme in place to frequently inspect, maintain, calibrate and repair equipment to ensure maximum use through its lifetime.

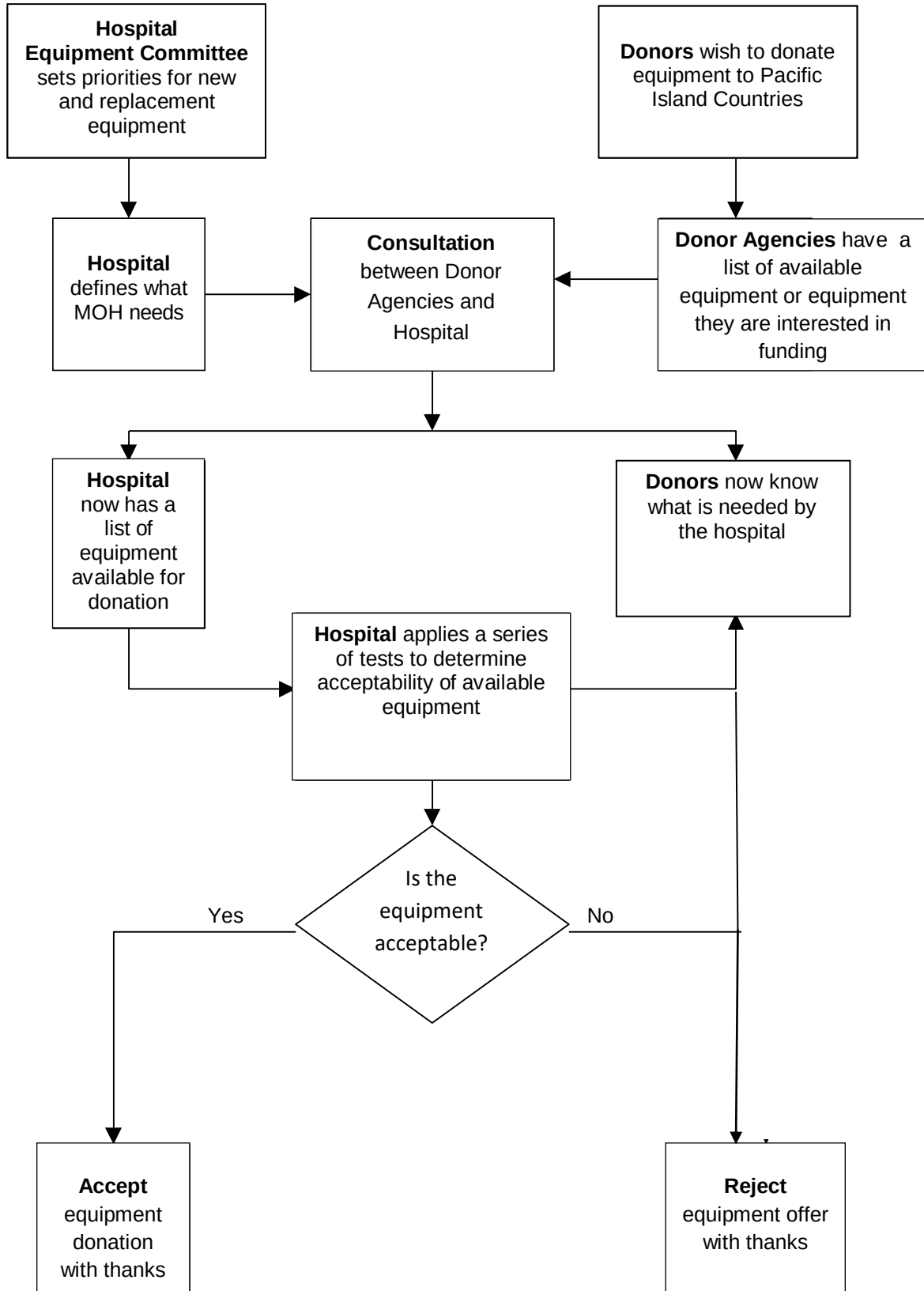
13.0 Follow-up Evaluation

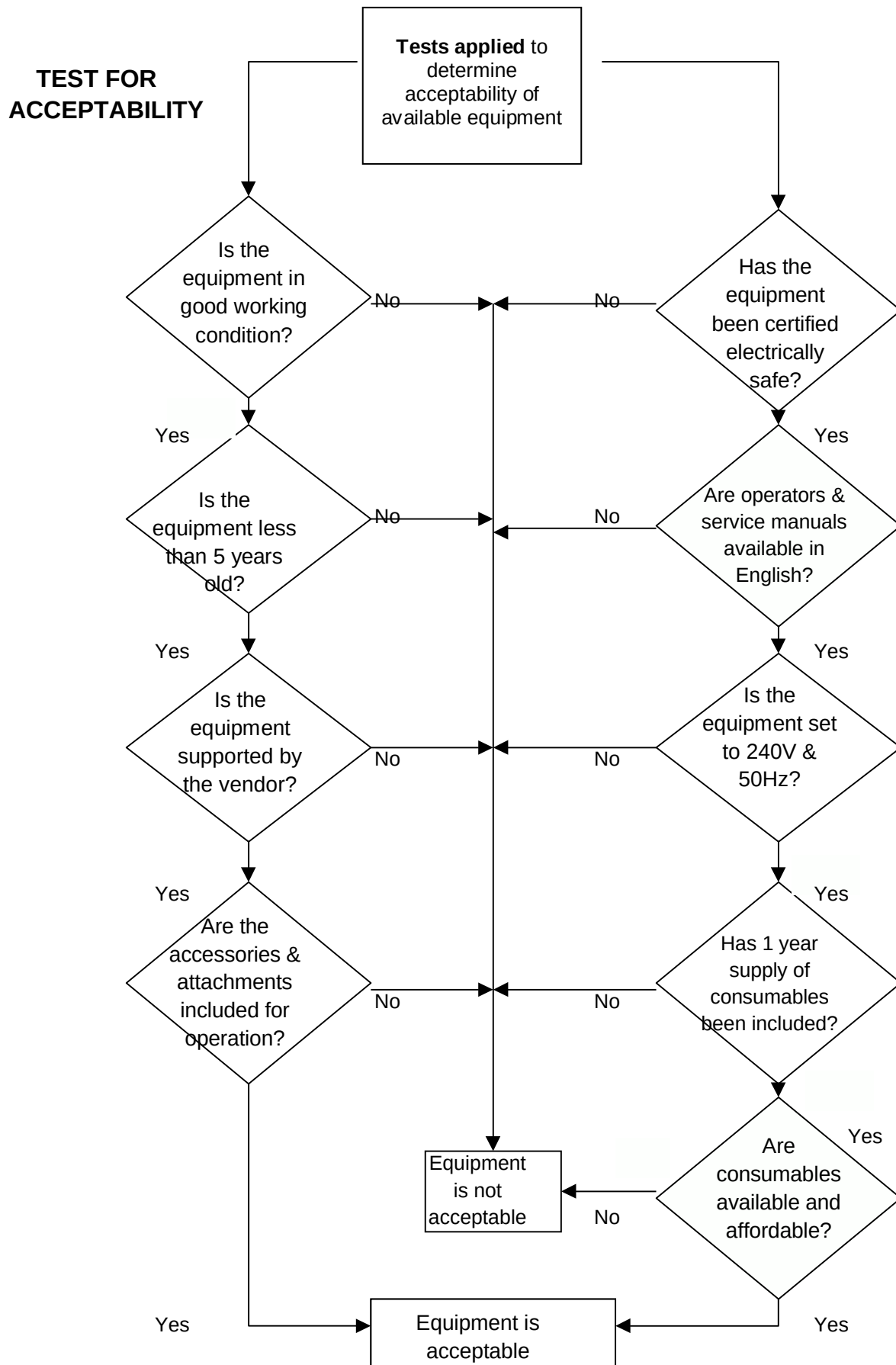
Both Donor and *Ministry/Department of Health* shall evaluate the success or failure of donation to improve future success rates.

14.0 Unserviceable Equipment

Standard procedures for writing off and disposal of fixed assets as defined in *Ministry/Department of Health/Finance Asset Management Manual/Regulations* shall be used to write-off and dispose any unserviceable equipment.

PROCESS FOR GIVING AND RECEIVING DONATIONS





15.0 Donation Checklist

To be completed by *Ministry/Health Department* in consultation with the donor, the end user and Equipment Committee (where applicable). Mark the applicable box for each line.

Equipment details	Donor details
Make	Contact name
Model	Organisation
Description	Address
Quantity	Email
	Phone

****Fill out for each donated item***

Need	Yes	No	NA
Is this equipment a genuine need for the country?			
Is this a priority for the country?			
Does the donation align with the strategic clinical services plans of the <i>Ministry/Health Department</i> ?			
If no real need, then decline the offer.			
Costs			
Does the donation have all the necessary accessories/consumables and components required for proper functionality? (IE if a bed, does it have a mattress? Or if an ultrasound machine, does it have all the transducers? Etc.) If incomplete provide details and costs here:			
Does the equipment require any changes to building infrastructure such as special gas, steam, cooling, power, water quality, shielding? If so, provide details here:			
Compliance			
Power – Does it operate on 240VAC 50Hz electrical? Type 1 plug top Safety – Does it conform to AS3551:2012? Standardization – Does the donation complement existing equipment. Will it be necessary to train clinicians in the use of this equipment? If it is weight bearing does it conform to MOH standards. (for beds/stretchers/maternity tables minimum 250kg, scales 200kg.			

Support			
Does the equipment meet safety, compliance, and expected useful life and appropriateness standards for Samoa? (including OHS&W and Environmentally Friendly) 240VAC 50Hz electrical?			
Is the equipment new?			
Is there any applicable warranty? If so, then provide details.			
If not new, then how old is the equipment? If the donation is over half of its life expectancy, then decline the offer. IE ECG machine is 7 years old but lifespan expectancy is 10 years.	Age in years:		
Is this model still supported by the manufacturer? Usually minimum 5 years from commissioning of goods			
Can biomedical engineering services maintain this equipment in-house with existing resources?			
Will outsourcing of support be required?			
Are there user manuals provided in English?			
Are there technical manuals provided in English?			
Shipping			
Will there be any shipping costs for the <i>Ministry/Department of Health</i> ? If so, provide details here: of all freight/shipping, landed costs IE customs duty and VAGST, and local handling fees.			
Total estimated cost to <i>Ministry/Department of Health</i> of acceptance of this donation			
Accept equipment? If no, give reasons: Donor: _____ Date: _____ MOH Rep: _____ Date: _____			
If donation is to be accepted , then proceed below.			

Documentation			
Do shipping documents state that it is a donation?			
Are all accompanying consumables, accessories, spare parts and manuals labelled accordingly?			
In the case of a refurbished item, is there documentation that states its condition is fit and safe for purpose? Please provide documentation			
Do shipping documents state the contents of the packaging/container?			
Hazardous items			
Does the equipment contain any potentially infectious material? If so, give details of decontamination performed.			
Does the equipment contain any radioactive source? If so, is it packaged in accordance with international standards?			
Customs Clearance and Unpacking			
Has any special documentation for assistance with Customs clearance been requested from the donor in advance? If so, give details:			
Equipment complete and all accessories accounted for. Additional comments: Head of Biomed Signed: _____ Date: _____			

All packages shall be unpacked and checked for damage and contents as per packing list. Any irregularities or missing items shall be reported to the donor **immediately**.

REFERENCES

1. Strengthening Specialized Clinical Services in the Pacific, Donation Policy, 2014
2. World Health Organisation (WHO), *Medical device donations: considerations for solicitation and provision*, June 2011
3. World Health Organisation (WHO), *Guidelines For Health Care Equipment Donations*; March 2000
4. Royal Australian College of Surgeons - Medical Equipment Maintenance Project For The Pacific, *Guidelines For The Donation Of Medical Equipment to the Pacific Island Countries - A guide for Donors and Recipients*.
5. Ministry of Health Samoa, Medical Equipment Donation Policy, 2019